Request for Information: Health Technology Ecosystem

Centers for Medicare & Medicaid Services (CMS)
Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC)
Department of Health and Human Services (HHS)

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On behalf of Element, I am pleased to submit our team's response to the Request for Information; Health Technology Ecosystem. This response incorporates both our team's professional perspective, based upon the work that Element does today, as well as my personal perspective as a primary healthcare coordinator for my father.

Element is an SBA Certified HUBZone IT services consulting firm dedicated to providing digital and innovation services to the U.S. Federal Health and Civilian agencies. We have over 15 years of experience delivering scalable and innovative digital services to federal and commercial customers. We have current work and past performance in the following areas of CMS:

- Center for Clinical Standards and Quality (CCSQ)
  - Quality Payment Program (QPP) Clinician Interface and Scoring (CIS)
  - End Stage Renal Disease Quality Reporting System Scoring & Feedback (EQRS S&F)
  - HCQIS Enterprise Integrated Supplemental Testing (HEIST)

- Center for Consumer Insurance Information and Oversight (CCIIO)
  - Marketplace Systems Integration Healthcare.gov (MSI)

- Center for Medicaid and Chip Services (CMCS)
  - Medicaid Drug Program (MDP)

- Office of Enterprise Data & Analytics (OEDA)
  - CMS Blue Button 2.0 (BB2.0) & Beneficiary FHIR Data Server (BFD)
  - Beneficiary Claims Data (BCDA) & Data the Point of Care (DPC)

We believe that our unique point of view will provide the "bold steps to modernize the nation's digital health ecosystem with a focus on empowering Medicare beneficiaries through greater access to innovative health technologies". Please find our comments below.

B. Patients and Caregivers

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve patients and caregivers. While we certainly want patients and caregivers to answer questions in this section (and in other sections) from the patient/caregiver point of view, we also invite all stakeholders to provide their viewpoints on the patient/caregiver workflows.

1. Patient Needs

PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

> *a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?*
>
> *b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.*

My father was diagnosed with Alzheimer's in 2024. I have attempted to coordinate and track his care remotely, although only about 50 miles away. I have a full medical power of attorney for my father. I would like to have a 360 degree view, and in some cases, insight in his care, appointments, medication, prior to them being presented to him, so I can ensure I have discussed, and prepared, both myself, and him for what is to be expected at his appointments.

PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

No. Currently, access to information is fragmented across various portals, and even aggregation platforms have significant gaps in their capabilities / don't generally allow for the re-export of EHI that has been imported.

a. If so, what are some examples of benefits it has provided?

b. If not, in what contexts or for what workflows would it be most valuable to use one portal or system to access all such health information?

c. Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?

Imaging information is overwhelmingly unavailable via standards-based APIs, regardless of context.

PC-3. Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?

PC-4. What features are missing from apps you use or that you are aware of today?

a. What apps should exist but do not yet? Why do you believe they do not exist yet?

b. What set of workflows do you believe CMS is uniquely positioned to offer?

CMS is uniquely positioned to be the "glue" for Medicare beneficiaries. According to a 2022 abstract (https://www.ajmc.com/view/turnover-among-new-medicare-advantage-enrollees-may-be-greater-than-perceived), ~50% of newly-enrolled Medicare Advantage enrollees will switch plans within 5 years. CMS can and should enable the consolidation of a beneficiary's healthcare information and consent/data sharing preferences (to include caregiving). Plans and apps may come and go, but the fact that very few individuals actually lose their Medicare entitlement means that better leveraging the medicare.gov account and CMS APIs can provide continuity for beneficiaries, regardless of which plan they're on.

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

Above all, CMS should ensure that any application potentially reviewed by CMS meets a baseline for patient privacy. Namely, Medicare beneficiaries should be able to demand their data is deleted from any application that uses their information. Existing standards of conduct (e.g. the CARIN Alliance Code of Conduct) only mandate the existence of a policy regarding data retention, not guarantee that a right to delete their data exists. This is critical to level the playing field between consumers and application developers by providing recourse for beneficiaries against applications that do not provide significant value.

b. What changes would enable timely access to high quality CMS and provider generated data on patients?

The ability to utilize digital identities to aggregate information from both CMS + external sources, in combination with service-level agreements for the time at which a piece of information must be made available after it is recorded (data retention minimum SLAs) would enable timely access to high quality data.

PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

Any products that appear in the CMS ecosystem should allow for the use of the medicare.gov login for single sign on. Additionally, if digital health products are associated with certain conditions, CMS should utilize their existing knowledge of a beneficiary's conditions (such as via risk adjustment data) to prioritize opportunities that are presented to a beneficiary. For example, it wouldn't make sense to show stroke recovery applications to individuals who have not had a stroke. This is important to prevent information overload.

PC-7. If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

Most digital health apps that claim to affect outcomes are overwhelmingly based upon diversion from emergency room visits. If CMS were to collect real-world data to that effect, this would likely be the most feasible first use case to evaluate. CMS already denies payment for preventable re-admissions, so there is existing institutional competence in evaluating similar metrics.

2. Data Access and Integration

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

In general, basic information such as USCDI data + claims information is available for Medicare beneficiaries, and is (generally) valuable to them.

a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?

Beyond specific categories of data, access for caregivers to data.

b. What are specific sources, other than claims and clinical data, that would be of highest value, and why?

Pricing information, especially as it relates to an individual's coverage information would be the most valuable to a consumer.

c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

There are many specific challenges regarding integration of data from different sources: Even with provenance information, secondary providers are hesitant to incorporate data from other sources into the "medical record"

As data is more freely exchanged, identification of duplicate records becomes an issue, especially if provenance information is not retained as data is re-transmitted.

PC-9. Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

All of these are incorporated below, but risk adjustment data and prior authorization are two valuable sources of information. Additionally, inclusion of high-quality part C data would be highly beneficial. Any information that is able to be manipulated or surfaced via medicare.gov, such as appointed representatives, self-reported medication lists, and choice of primary care doctor, are valuable pieces of information.

a. What difficulties are there in accessing or utilizing these data sources today?

Since CMS-0057F doesn't require prior authorization information to be available via Patient Access APIs until 1/1/27, the quality of claims data is extremely disparate across payers managing Part C plans.

b. What suggestions do you have to improve the Blue Button 2.0 API experience?

- Lead by example by enabling a version of the patient-data-feed from US Core, but for claims data (https://build.fhir.org/ig/HL7/US-Core/patient-data-feed)
- Enable access to discrete data elements outside of the ExplanationOfBenefit resource - for example, deductible information, lifetime reserve days used, cost

information, etc.

- Expose Medicare prior authorization information via the Blue Button 2.0 API
- Expose Primary Care Provider selections from medicare.gov using FHIR (Patient.generalPractitioner)
- Expose a Beneficiary's listed medication list from medicare.gov (using MedicationRequest)
- Expose Medicaid claims information
- To enable the administration's goal of managing chronic conditions, CMS APIs should expose risk adjustment information via Condition resources. Since CMS adjusts capitation payments on the basis of recorded diagnoses, this provides a unique opportunity to enable applications that manage chronic conditions access to a source of truth with regards to an individual's chronic conditions.
- Enable more granular control of access to information (currently, consent with Blue Button 2.0 is mostly all-or-nothing. The only level of granularity is to allow access to all information, all information minus the Patient resource, or no information at all. All resource permutations should be allowed).
- Enable caregivers to get access to data - medicare.gov does not allow for sharing of accounts, which theoretically makes using Blue Button 2.0 impossible for certain individuals, such as those with severe disabilities due to stroke (and there is a high overlap between these individuals, and those who could benefit the most from access to comprehensive data!)

c. Is there non-CMS data that should be included in the API?

CMS should allow for patients to annotate information in their record as patient-generated health data. CMS is uniquely positioned to enable this given that it is extremely unlikely for a Medicare enrollee to stop utilizing their entitlement once they gain it (regardless of whether they participate in Medicare Advantage or Medicare Fee For Service).

PC-10. How is the Trusted Exchange Framework and Common AgreementTM (TEFCATM) currently helping to advance patient access to health information in the real world?

TEFCA is still in its infancy, and there is significant opportunity for improvement.

CMS and ASTP/ONC should ensure that advancements in the maturity of TEFCA do not come at the expense of existing capabilities in the Patient Access API space. That would significantly harm innovation due to the fees required to join a QHIN, even for IAS use cases.

In general, the "claims" used for the FAST Security workflow need to be formalized for use beyond TEFCA, since the existing literature of using identity credentials in the context of SMART is limited to the TEFCA Facilitated FHIR IG. This will be critical if CMS is to participate in "Trusted Exchange" outside of TEFCA.
TEFCA also disproportionately stands to benefit payers and those who commercialize data rather than patients, given the non-existence of a capability for individuals to pre-emptively prevent sharing of their data via TEFCA.

a. Please provide specific examples.

b. What changes would you suggest?

c. What use cases could have a significant impact if implemented through TEFCA?

d. What standards are you aware of that are currently working well to advance access and existing exchange purposes?

e. What standards are you aware of that are not currently in wide use, but could improve data access and integration?

f. Are there redundant standards, protocols, or channels that should be consolidated?

g. Are there adequate alternatives outside of TEFCA for achieving widespread patient access to their health information?

PC-11. How are health information exchanges (HIEs) currently helping to advance patient

access to health information in the real world?

a. How valuable, available, and accurate do you find the data they share to be?

b. What changes would you suggest?

c. Are there particular examples of high-performing HIE models that you believe should be propagated across markets?

d. What is the ongoing role of HIEs amidst other entities facilitating data exchange and broader frameworks for data exchange (for example, vendor health information networks, TEFCA, private exchange networks, etc.)?

PC-12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

a. Examples may include the following:
(1) Binding cost estimates for pre-defined periods.
(2) Viewing provider schedule availability.
(3) Using third-party apps for appointment management.
(4) Accessing patient-facing quality metrics.
(5) Finding the right provider for specific healthcare needs.

b. What use cases are possible today?

c. What should be possible in the near future?

d. What would be very valuable but may be very hard to achieve?

All of the example use cases in this question are valuable. As several lawsuits against EHRs for information blocking indicate (see: CureIS vs. Epic, PointClickCare vs. RTMS), the primary impediment is a lack of available APIs to enable scheduling + other functionalities that have been traditionally siloed within an EHR / EHR portal.

Addressing key operational health data challenges can enhance care navigation and

reduce barriers to competition among providers. Several critical use cases stand out, including binding cost estimates for pre-defined periods, which would provide patients with accurate and predictable financial information, improving decision-making and price transparency. Viewing provider schedule availability in real time would allow patients to access care more efficiently, reducing delays and streamlining appointment scheduling across networks. The use of third-party apps for appointment management would enable seamless scheduling, offering convenience while fostering provider competition through greater accessibility. Accessing patient-facing quality metrics would empower individuals to make informed choices based on transparent performance data, encouraging providers to enhance service quality. Finding the right provider for specific healthcare needs through AI-driven search tools would refine provider matching based on medical history and preferences, ensuring patients receive the most suitable care while promoting equitable access.

Many of these use cases are possible today, though they often lack full integration or standardization. Cost estimation tools exist but are rarely binding, leaving patients uncertain about final expenses. Some providers offer online scheduling, yet interoperability across systems remains limited. Third-party applications allow appointment booking, but many lack real-time synchronization with provider availability. Quality metrics are published on various platforms, but inconsistencies in data collection and presentation make it difficult for patients to compare providers effectively. AI-driven provider matching is available but relies on incomplete datasets, limiting accuracy.

In the near future, advancements could enable standardized binding cost estimates through AI models trained on pricing trends, delivering reliable financial projections. Full scheduling interoperability would allow seamless coordination across providers, ensuring availability is visible across healthcare networks. Expanded third-party app functionality with open APIs would enable comprehensive appointment management across all providers, improving accessibility and efficiency. User-friendly, standardized quality metrics integrated with dynamic patient feedback and real-time performance tracking would enhance decision-making. AI-enhanced provider selection could offer personalized recommendations based on treatment outcomes, individual health data, and patient preferences, further refining care navigation.

While these innovations hold significant promise, certain advancements would be challenging to achieve due to regulatory constraints, technological limitations, and industry

3. Information Blocking and Digital Identity

PC-13. How can CMS encourage patients and caregivers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would the impact? Would increasing reporting of complaints advance or negatively impact data exchange?

- CMS should publicly post the results of information blocking complaints. It will absolutely advance data exchange, as specific details will help developers understand what is / isn't allowed. As it stands today, it is unclear what percentage of information blocking complaints are fully investigated / what actions are taken. Showing that the system is enforced will encourage its use, rather than enabling the bystander effect.

PC-14. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 credentialing service providers (CSP))

a. What are the challenges to using existing technologies to improve patient digital identity credentials?

b. What could be the benefits to patients/caregivers if digital identity credentials were more widely used?

- Reduce the role of knowledge-based-authentication, which has been inappropriately used on a widespread basis to elevate identities to IAL2. Additionally, given that CMS' does explicitly endorse the sharing of medicare.gov login credentials, the combination of acceptance of digital identities and a digital representation of authorized representatives

opens up the potential for a secure means of caregiver access to information via FHIR APIs.

c. What are the potential downsides?

Fallback methods of identity verification will be needed, since CSP's cannot guarantee that 100% of individuals will be able to be easily verified. As well, there is not currently a standard for re-using an ID token issued by a CSPs in the SMART App Launch Implementation guide as part of the authorization flow. This concept only currently exists in the TEFCA Facilitated FHIR IG, limiting its use to those who pay to join a QHIN. The FAST B2C implementation is not enough to level the playing field between TEFCA and non-TEFCA IAS. This limits innovation in the space of patient access APIs.

d. How would encouraging the use of CSPs improve access to health information?

Access to health information will become more ubiquitous if the ease of accessing it for individuals is improved, especially if a single ID token is able to be re-used across systems.

e. What role should CMS/payers, providers, and app developers have in driving adoption?

CMS should be a leader in the adoption of digital identity credentials. There are several methods this could take, from respecting a digital credential passed by an application as part of TEFCA-based exchange, to generating an appropriate digital identity based upon a medicare.gov login to initiate a pull of information from other portals (a la Tiered OAuth).

f. How can CMS encourage patients to get digital identity credentials?

Subsidizing costs associated with their usage, and enabling Medicare beneficiaries to get digital identity credentials using their medicare.gov credentials.

C. Providers

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve providers. While we certainly want providers to answer questions in this section (and in other sections) from the provider point of view, we also invite all stakeholders to provide their viewpoints on the provider workflows as appropriate.

1. Digital Health Apps

PR-1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

> *a. What are the current obstacles?*
> *b. What information should providers share with patients when using digital products in the provision of their care?*
> *c. What responsibilities do providers have when recommending use of a digital product by a patient?*

PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

PR-3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)?

Please address all of the following:

> *a. Current challenges in accessing different data formats.*
> *b. Impact on patient care quality.*
> *c. Technical barriers to full data accessibility.*
> *d. Cost or privacy implications of making all data formats interoperable.*
> *e. Priority level compared to other interoperability needs.*

Ensuring that all data within an electronic health record (EHR) system is accessible for exchange, regardless of its storage format, is critical for improving healthcare delivery and interoperability in both urban and rural areas. The ability to seamlessly exchange structured data fields, scanned documents, faxed records, lab results, and free-text notes enhances care coordination, supports informed clinical decisions, and ultimately improves patient outcomes.

Currently, one of the biggest challenges in accessing different data formats lies in the lack of standardization across EHR systems. Scanned documents and faxed records often exist as static images, making them difficult to search or extract meaningful insights from. Free-text notes, while rich in clinical detail, require advanced natural language processing (NLP) tools to standardize and integrate effectively. Lab results and structured data fields are more interoperable but are often siloed within specific vendor platforms, limiting their exchangeability across healthcare networks.

The impact on patient care quality is significant. Limited access to a patient's complete history can lead to fragmented care, repeated tests, and delayed diagnoses. When providers cannot easily retrieve past medical records, they may miss critical insights that influence treatment plans. Interoperability across all data types would ensure continuity of care, enabling better-informed decision-making and reducing unnecessary procedures.

There are substantial technical barriers to achieving full data accessibility. Converting scanned documents and faxed records into structured, interoperable formats requires optical character recognition (OCR) and AI-driven data extraction tools, which can be complex to implement. Free-text notes necessitate advanced NLP to convert unstructured information into standardized formats. Legacy EHR systems may lack the capability to integrate such technologies, further complicating efforts to achieve seamless interoperability.

Cost and privacy concerns are also key considerations. Making all data formats interoperable requires investment in infrastructure, AI tools, and compliance with regulatory standards such as HIPAA. Protecting sensitive information in scanned documents and unstructured notes adds complexity, as automated data extraction must balance usability with privacy safeguards. Cybersecurity risks also increase with expanded data sharing, necessitating robust encryption and access controls.

In terms of priority level, full data accessibility ranks highly among interoperability needs, but it must be balanced with other pressing initiatives, such as real-time patient data exchange, cross-platform system integration, and standardized health information exchanges (HIEs). While ensuring interoperability across all formats would yield immense benefits, addressing fundamental interoperability gaps in structured data exchange may offer more immediate improvements in healthcare efficiency.

PR-4. What changes or improvements to standards or policies might be needed for patients' third-party digital products to have access to administrative workflows, such as auto-populating intake forms, viewing provider information and schedules, and making and modifying an appointment?

2. Data Exchange

PR-5. Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

a. Patient Access API API
b. Standardized API for Patient and Population Services
c. Provider Directory API
d. Provider Access API
e. Payer-to-Payer API
f. Prior Authorization API
g. Bulk FHIR – Do you support Group ID-based access filtering for population-specific queries?
h. SMART on FHIR – Do you support both EHR-launched and standalone app access? What does the process for application deployment entail?
i. CDS Hooks (for clinical decision support integrations)

PR-6. Is TEFCA currently helping to advance provider access to health information?

a. Please provide specific examples.
b. What changes would you suggest?
c. What other options are available outside of TEFCA?
d. Are there redundant standards, protocols or channels or both that could be consolidated?

PR-7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

PR-8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

a. What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?

The use of Bulk FHIR (Fast Healthcare Interoperability Resources) data exports from Electronic Health Records (EHRs) to CMS presents several advantages and challenges:

Benefits:

- Automation & Efficiency: Bulk FHIR exports streamline data submission by automating the extraction and transfer of structured clinical quality data, reducing manual reporting burdens.
- Standardization: FHIR-based data exchange ensures consistency across different EHR systems, improving data integrity and interoperability.
- Timeliness: Real-time or near-real-time data availability enhances CMS's ability to assess quality metrics promptly.
- Reduced Provider Burden: If CMS assumes responsibility for quality metric calculations, providers can focus more on patient care rather than administrative tasks.

Downsides:

- Implementation Complexity: Transitioning to Bulk FHIR requires significant investment in infrastructure and compliance with evolving standards. However, all EHRs have experience in related concepts.
- Data Accuracy & Completeness: Ensuring that exported data accurately reflects clinical quality measures without manual validation may pose challenges.

CMS can reduce provider burden by leveraging Bulk FHIR exports to automate quality metric calculations, minimizing the need for manual data entry and reconciliation.

b. In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually purposed?

To dual-purpose investments and streamline provider responsibilities, CMS can:

- Integrate Quality Reporting with Interoperability Initiatives: Aligning quality reporting program requirements with broader interoperability mandates (e.g., TEFCA) ensures that data exchange efforts serve multiple purposes.
- Enhance API-Based Data Sharing: Encouraging the adoption of FHIR-based APIs allows providers to submit quality data seamlessly while also supporting clinical decision-making.
- Leverage Existing Health Information Exchanges (HIEs): Utilizing HIEs as intermediaries for quality reporting can reduce redundancy and improve data accessibility.

c. Are there requirements CMS should consider for data registries to support digital quality measurement in a more efficient manner? Are there requirements CMS should consider for data registries that would support access to real-time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

To enhance efficiency and real-time access to quality data, CMS should consider:

- Standardized Data Formats: Mandating FHIR-based data structures for registries to ensure interoperability.
- Automated Data Validation & Feedback Loops: Implementing real-time validation mechanisms to improve data accuracy before submission.
- Provider Access to Actionable Insights: Ensuring registries provide real-time dashboards that allow providers to track performance and adjust care strategies accordingly.
- Security & Compliance Measures: Strengthening data governance policies to protect patient information while facilitating seamless reporting.

3. Digital Identity

PR-9. How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

a. What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider?

b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?

PR-10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs):

a. What are the challenges and benefits for providers?

b. How would requiring their use improve access to health information?

c. What are the potential downsides?

d. What impact would mandatory credentials have on a nationwide provider directory?

e. How could digital identity implementation improve provider data flow?

f. Would combining FHIR addresses and identity improve data flow?

PR-11. How could members of trust communities[3] (for example, QHINs, participants and subparticipants in TEFCA, which requires Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs)) better support the goals of reduced provider and patient burden while also enhancing identity management and security?

4. Information Blocking

PR-12. Should ASTP/ONC consider removing or revising any of the information blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?

PR-13. For any category of healthcare provider (as defined in 42 U.S.C. 300jj(3)), without a current information blocking disincentive established by CMS, what would be the most effective disincentive for that category of provider?

PR-14. How can CMS encourage providers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

D. Payers

PA-1. What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder participation in TEFCA?

The cost to join a QHIN and as-of-yet fully developed SOPs are the biggest limitations to TEFCA today. Joining a QHIN should be free for organizations/applications that solely use TEFCA for IAS.

PA-2. How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

CMS should publicly reaffirm the implementation date of CMS-0057F. CMS should also advance DPC from the pilot stage as soon as practicable in order to share insight on its rollout, as a reference for other provider access APIs.

PA-3. How can CMS encourage payers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins?

PA-4. What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

PA-5. What are ways payers can help with simplifying clinical quality data responsibilities of providers?

> *a. How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?*
>
> *b. In what ways can the interoperability and quality reporting responsibilities of providers to both CMS and other payers be consolidated so investments can be dually purposed? Are there technologies payers might leverage that would support access to real time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?*

PA-7. How can CMS encourage payers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

E. Technology Vendors, Data Providers, and Networks

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve technology vendors, data providers, and networks. While we certainly want technology vendors, data providers, and networks to answer questions in this

section (and in other sections) from their point of view, we also invite all stakeholders to provide their viewpoints on the technology vendor, data provider, and network use cases as appropriate.

1. Ecosystem

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

To stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers, the Centers for Medicare & Medicaid Services (CMS) can implement several short-term and longer-term strategies aimed at fostering innovation, reducing barriers to entry, and encouraging interoperability.

In the short term, CMS can focus on expanding access to standardized APIs to ensure developers can easily integrate new solutions with existing Medicare systems. By promoting the adoption of Fast Healthcare Interoperability Resources (FHIR) and other open standards, CMS can streamline data exchange, making it easier for developers to build digital tools that improve patient and caregiver experience. Additionally, offering financial incentives, such as grants or reimbursement models for digital health applications, would encourage startups and established vendors to prioritize Medicare-specific innovations. Simplifying regulatory compliance through clear guidance on security, data sharing, and patient privacy would also reduce friction for developers and accelerate the deployment of digital health solutions.

For longer-term impact, CMS can establish a structured innovation ecosystem, including a Medicare-focused technology sandbox where developers can test interoperability and usability of new tools without extensive compliance hurdles. Supporting AI-driven analytics and personalized care solutions through funding and policy frameworks would enable more sophisticated digital health applications tailored to Medicare beneficiaries. Encouraging public-private partnerships with technology firms, universities, and research institutions would foster continued investment and research into next-generation health solutions. CMS could also work toward broader interoperability mandates, ensuring seamless data access across providers, payers, and patient-facing applications to enhance care coordination and efficiency.

Taken together, these steps would create a more attractive environment for developers to build scalable, Medicare-friendly digital health solutions while improving accessibility, efficiency, and patient outcomes.

TD-2. Regarding CMS Data, to stimulate developer interest--

a. What additional data would be most valuable if made available through CMS APIs?

- Lead by example by enabling a version of the patient-data-feed from US Core, but for claims data (https://build.fhir.org/ig/HL7/US-Core/patient-data-feed)
- Enable access to discrete data elements outside of the ExplanationOfBenefit resource - for example, deductible information, lifetime reserve days used, cost information, etc.
- Expose Medicare prior authorization information via the Blue Button 2.0 API
- Expose Primary Care Provider selections from medicare.gov using FHIR (Patient.generalPractitioner)
- Expose a Beneficiary's listed medication list from medicare.gov (using MedicationRequest)
- To enable the administration's goal of managing chronic conditions, CMS APIs should expose risk adjustment information via Condition resources.
- Enable more granular access to information (currently, consent is mostly all-or-nothing. The only level of granularity is to allow access to all information, all information minus the Patient resource, or no information at all. All resource permutations should be allowed)
- Enable caregivers to get access to data - medicare.gov does not allow for sharing of accounts, which theoretically makes using Blue Button 2.0 impossible for certain individuals, such as those with severe disabilities due to stroke (and there is a high overlap between these individuals, and those who could benefit the most from access to comprehensive data!)

b. What data sources are most valuable alongside the data available through the Blue Button 2.0 API?

c. What obstacles prevent accessing these data sources today?

d. What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?

2. Digital Identity

TD-3. Regarding digital identity implementation:

a. What are the challenges and benefits?

b. How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?

c. What impact would mandatory use of the OpenID Connect identity protocol have?

3. Technical Standards and Certification

TD-4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

One of the most impactful approaches is mandating the use of open standards such as Fast Healthcare Interoperability Resources (FHIR) and incentivizing compliance through regulatory frameworks. By requiring healthcare providers and vendors to adhere to standardized API protocols for Medicare and Medicaid data exchange, CMS can drive industry-wide adoption while reducing fragmentation caused by proprietary solutions. Another critical step is providing financial incentives and support for developers who integrate open APIs into their digital health products. Offering grants, reimbursement programs, or development subsidies for applications that leverage standards-based APIs would make compliance economically attractive while ensuring continued innovation in patient-centered technology.

CMS can also develop robust technical guidance and compliance frameworks to simplify implementation for healthcare organizations and vendors. Many providers hesitate to adopt open APIs due to uncertainty regarding security, data governance, and usability. Clear documentation, sandbox testing environments, and certification programs can alleviate these concerns and accelerate adoption.

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

TD-6. What unique interoperability functions does TEFCA perform?

TEFCA uniquely positions itself as a record locator service. It serves as the testing ground for digital identities as part of the Connected FHIR IG, but it is imperative that it does not remain unique to TEFCA-based exchange.

a. What existing alternatives should be considered?

b. Are there redundant standards, protocols or channels or both that should be consolidated?

TD-7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

USCDI has improved interoperability by increasing the standardization of common data elements that are necessary for the utilization of health care records.

a. Does it contain the full extent of data elements you need?

No, Claims-related information should be included in the USCDI. This is a common comment on new versions of USCDI from the CARIN Alliance. Element agrees that CPCDS elements should be added to USCDI (https://build.fhir.org/ig/HL7/carin-bb/Common_Payer_Consumer_Data_Set.html). ASTP/ONC should also ensure that vocabularies present in and referenced by USCDI fully meet the needs of the use cases those data elements support. For example, SOPT does not fully enumerate all types of coverage for Medicare, leaving ambiguity between Part A and Part B.

b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized?

c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?

d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

No. While unstructured data elements may be usable with language models, it would be a mistake to accept "can work" with "ideal." To the contrary, additional structured data elements will only enable advancements in language models to better utilize that data.

TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

TD-9. Regarding certification of health IT:

a. What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?

b. What would be the drawbacks?

c. How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient's chart (for example, faxed records, free text, discrete data)?

d. What policy changes could CMS make so providers are motivated to respond to API-based data requests with best possible coverage and quality of data?

CMS could motivate providers to respond to API-based data requests with high-quality and comprehensive data by implementing several strategic policy changes. Financial incentives, such as enhanced reimbursement rates and performance-based bonuses, could encourage providers to adopt and maintain effective API-driven data exchange systems. Additionally, regulatory requirements mandating the use of standardized APIs like FHIR and integrating API-based data sharing into existing quality reporting programs would ensure consistency

and compliance. Providing technical support, including funding for infrastructure upgrades and enforcing uniform data standards, would help providers seamlessly implement API-compatible systems. Transparency measures, such as publicly reporting provider performance metrics and conducting regular audits, could further promote accountability. A patient-centered approach, ensuring individuals can easily access their health data via APIs and incorporating consumer feedback into provider ratings, would also create a strong motivation for providers to prioritize data quality and coverage. These policy adjustments would not only improve interoperability but also enhance the overall efficiency and effectiveness of healthcare data exchange.

e. How could EHRs capable of bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit? What concerns are there with this approach?

Electronic Health Records (EHRs)with bulk data transfer capabilities can significantly reduce the burden on providers when reporting quality performance data to the Centers for Medicare & Medicaid Services (CMS). Here's how:

Benefits

- Automated Data Submission: Bulk data transfer allows providers to send large volumes of patient data directly to CMS, eliminating the need for manual entry and reducing administrative workload.
- Improved Accuracy: Automation minimizes human errors in data reporting, ensuring more reliable quality performance metrics.
- Real-Time Reporting: Faster data exchange enables near real-time performance tracking, allowing providers to make timely improvements in care delivery.
- Interoperability: Standardized data formats, such as FHIR (Fast Healthcare Interoperability Resources), facilitate seamless integration across different health IT systems.

Capabilities

- FHIR-Based APIs: Adoption of FHIR Release 4 ensures structured and standardized data exchange.
- Data Aggregation Tools: Systems must be able to compile and format data

according to CMS reporting requirements.

- Security & Compliance: Robust encryption and access controls are necessary to protect patient data during transfer.
- Automated Quality Measure Calculation: Integration with Digital Quality Measures (dQMs) can streamline reporting.

Concerns & Challenges

- Data Privacy & Security: Bulk transfers involve sensitive patient information, requiring strict compliance with HIPAA and CMS regulations.
- Technical Barriers: Some providers may lack the infrastructure to support bulk data exchange, leading to disparities in adoption.
- Standardization Issues: Variability in EHR systems may cause inconsistencies in data formatting and interpretation.
- Implementation Costs: Upgrading systems to support bulk data transfer may require significant investment

TD-10. For EHR and other developers subject to the ONC Health IT Certification Program, what further steps should ASTP/ONC consider to implement the 21st Century Cures Act's API condition of certification (42 U.S.C. 300jj-11(c)(5)(D)(iv)) that requires a developer's APIs to allow health information to be accessed, exchanged, and used without special effort, including providing access to all data elements of a patient's electronic health record to the extent permissible under applicable privacy laws?

TD-11. As of January 1, 2024, many health IT developers with products certified through the ONC Health IT Certification Program are required to include the capability to perform an electronic health information export or "EHI export" for a single patient as well as for patient populations (45 CFR 170.315(b)(10)). Such health IT developers are also required to publicly describe the format of the EHI export. Notably, how EHI export was accomplished was left entirely to the health IT developer. Now that this capability has been in production for over a year, CMS and ASTP/ONC seek input on the following:

a. Should this capability be revised to specify standardized API requirements for EHI export?

Yes. Unfortunately, this requirement has been interpreted by some EHRs as "self-service CCD export," which is woefully inadequate. ASTP/ONC should clarify the expected payload format and not leave it up to individual health IT developers, to also include a standardized API.

b. Are there specific workflow aspects that could be improved?

c. Should CMS consider policy changes to support this capability's use?

4. Data Exchange

TD-12. Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

CMS should not explicitly endorse non-CMS data sources and networks.

TD-13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

a. What are the primary obstacles to this?

b. What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?

TD-14. Regarding networks' use of FHIR APIs:

a. How many endpoints is your network connected to for patient data sharing? What types, categories, geographies of endpoints do you cover? Are they searchable by National Provider Identifier (NPI) or organizational ID?

b. How are these connections established (for example, FHIR (g)(10) endpoints, TEFCA/Integrating the Health Enterprise (IHE) XCA, or proprietary APIs)?

c. Do you interconnect with other networks? Under what frameworks (for example,

TEFCA, private agreements)?

TD-15. Regarding bulk FHIR APIs:

Before answering the two sub questions, it's important that CMS and ASTP/ONC be skeptical of criticism of Bulk FHIR, either as surfaced in this RFI or in alternate channels. Problems relate not to the specification, but rather a choice by some implementers to under-invest in performance and functionality of their approach/architecture. CMS and ASTP/ONC should mandate the following for Bulk FHIR APIs:

1. Mandate support for the _since and _typeFilter parameters. These parameters are crucial for scalable use of Bulk FHIR. For _typeFilter, Bulk FHIR APIs SHALL support query parameters supported by a system's Patient Access API. If there is no equivalent Patient Access API, then search parameters that are common to all resources (such as _tag, etc), as well as those listed in US Core shall be the bare minimum for each applicable resource.
2. SLAs for the performance of Bulk APIs, that are benchmarked against CMS' own capabilities. CMS leads by example with Bulk FHIR, and others that are required to support Bulk FHIR should be able to have at least some percentage of the throughput that the government has already demonstrated possible. CMS itself offers Bulk FHIR APIs (such as BCDA) that offer throughput of over 5x the highest performing Bulk FHIR API analyzed in a published article. Compared to one of the EHRs compared and based upon our own testing, CMS' own APIs can offer 20x the throughput. https://pmc.ncbi.nlm.nih.gov/articles/PMC10593080/

a. How would increased use of bulk FHIR improve use cases and data flow?

Increased use of bulk FHIR would lower the barrier to entry for innovative organizations and startups who wish to utilize FHIR data and integrate via backend-systems integrations. Increased use of Bulk FHIR, including in CMS APIs can also help CMS save on costs - by being able to target queries for export, storage costs + execution time can be reduced.

b. What are the potential disadvantages of their use?

It is important to be selective in which use cases adopt bulk FHIR, and how it is adopted. For example, based upon some questions in this RFI, it sounds like CMS is considering requiring EHRs to perform bulk exports, which are then imported to CMS, for CMS to perform quality calculations. It would be significantly more cost-efficient for taxpayers for quality measure execution to be performed locally (eg at the EHR level), with the resulting MeasureReports being exported and sent in bulk to CMS for reporting.

TD-16. What are the tradeoffs of maintaining point-to-point models vs. shared network infrastructure?

a. Do current rules encourage scalable network participation?

b. What changes would improve alignment (for example, API unification, reciprocal access)?

TD-17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

CMS should actively participate in healthcare data sharing networks when it is determined that it would improve the efficiency of CMS' operations. Networks should be self-sufficient and not dependent upon continued funding from CMS or ASTP/ONC.

5. Compliance

TD-18. Information blocking:

a. Could you, as a technology vendor, provide examples for the types of practices you have experienced that may constitute information blocking. Please include both situations of non-responsiveness as well as situations that may cause a failure or unusable response?

b. What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices?

Public accountability measures and significant financial penalties.

c. Are there specific categories of healthcare actors covered under the definition of information blocking in section 3022(a)(1) of the Public Health Service Act (PHSA) that lack information blocking disincentives?

TD-19. Regarding price transparency implementation:

a. What are current shortcomings in content, format, delivery, and timeliness?

b. Which workflows would benefit most from functional price transparency?

c. What improvements would be most valuable for patients, providers, or payers, including CMS?

Price transparency information should be available at a standardized endpoint, rather than the practical implementation today of navigating a maze.

d. What would further motivate solution development?

A significant reduction in the fee schedule paid to providers/organizations not in compliance with price transparency.

F. Value-Based Care Organizations

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve value-based care organizations. While we certainly want value-based care organizations to answer questions in this section (and in other sections) from the value-based care provider point of view, we also invite all stakeholders to provide their viewpoints on the value-based care workflows as appropriate.

1. Digital Health Adoption

VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

Encouraging accountable care organizations (ACOs) and participants in the Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products requires a combination of financial incentives, regulatory support, and technological improvements that reduce barriers to adoption while maximizing efficiency and patient engagement.

One of the strongest incentives would be enhanced financial rewards for digital care integration, such as adjustments to shared savings calculations that recognize improvements in patient engagement and care coordination through digital tools. CMS could also establish targeted reimbursement models for providers who effectively deploy telehealth, remote monitoring, and AI-driven care navigation solutions. Another incentive would be performance-based funding that ties additional resources to demonstrated improvements in patient outcomes achieved through digital tools. For example, organizations that successfully reduce hospital readmissions or improve chronic disease management through digital health strategies could qualify for greater shared savings or additional funding.

Regulatory incentives, such as reduced administrative burdens for digitally enabled ACOs, could also drive adoption. Streamlining compliance requirements for organizations that utilize interoperable digital health management solutions would encourage more providers to integrate these technologies into their workflows. Additionally, CMS could support interoperability mandates that make it easier for ACOs to connect digital tools across provider networks, ensuring seamless patient data exchange.

Despite these potential incentives, several obstacles continue to hinder broader adoption. A significant challenge is interoperability limitations, as many existing health IT systems do not seamlessly integrate with third-party digital care products. Without standardized APIs and

data-sharing frameworks, ACOs struggle to incorporate digital solutions into their existing infrastructure. Cost and resource constraints also pose barriers—while digital health products may offer long-term savings, initial implementation costs, training efforts, and workflow disruptions deter organizations from early adoption. Provider and patient engagement is another challenge, as many stakeholders are unfamiliar with or resistant to transitioning from traditional care models to digitally enhanced solutions. Without proper education and incentives, adoption remains slow.

CMS could address these barriers by offering funding support for health IT modernization, promoting user-friendly digital solutions, and expanding data-sharing initiatives to enhance interoperability. By aligning financial incentives, regulatory frameworks, and technical infrastructure with digital care models, ACOs and MSSP participants would be better positioned to integrate these technologies for more efficient and effective patient care.

VB-2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

VB-3. What are essential health IT capabilities for value-based care arrangements?

> *a. Examples (not comprehensive) may include: care planning, patient event notification, data extraction/normalization, quality performance measurement, access to claims data, attribution and patient ID matching, remote device interoperability, or other patient empowerment tools.*
>
> *b. What other health IT capabilities have proven valuable to succeeding in value-based care arrangements?*

VB-4. What are the essential data types needed for successful participation in value- based care arrangements?

2. Compliance and Certification

VB-5. In your experience, how do current certification criteria and standards incorporated into the ONC Health IT Certification Program support value-based care delivery?

VB-6. What specific health information technology capabilities that could benefit APMs are not currently addressed by existing certification criteria and standards that should be included under the ONC Health IT Certification Program?

VB-7. How can technology requirements for APMs, established through CEHRT or other pathways, reduce complexity while preserving necessary flexibility?

VB-8. How can other HHS policies supplement CEHRT requirements to better optimize the use of digital health products in APMs? As an example, requirements under the Conditions of Participation for hospitals (42 CFR 482.24(d)) require hospitals to transmit electronic patient event notifications to community providers. What barriers are in place preventing APM participants from receiving the same notifications?

VB-9. What technology requirements should be different for APM organizations when comparing to non-APM organizations (for example, quality reporting, and interoperability)?

VB-10. In the Calendar Year (CY) 2024 Physician Fee Schedule final rule (88 FR 79413), CMS established that CEHRT requirements for Advanced APMs beyond those in the "Base EHR" definition should be flexible based on what is applicable to the APM that year based on the area of clinical practice. What certification criteria should CMS identify under this flexibility for specific Advanced APMs, or for Advanced APMs in general? Are there specific flexibilities or alternatives to consider for smaller or resource-constrained (such as rural) providers in meeting CEHRT requirements without compromising quality of care or availability of performance data?

3. Technical Standards

VB-11. What specific interoperability challenges have you encountered in implementing value-based care programs?

VB-12. What technology standardization would preserve program-specific flexibility while promoting innovation in APM technology implementation?

VB-13. What improvements to existing criteria and standards would better support value-based care capabilities while reducing provider burden?

VB-14. How could implementing digital identity credentials improve value-based care delivery and outcomes?

VB-15. How could a nationwide provider directory of FHIR endpoints help improve access to patient data and understanding of claims data sources? What key data elements would be necessary in a nationwide FHIR endpoints directory to maximize its effectiveness?

Conclusion

With over 15 years of delivering innovative digital services to federal agencies, including significant work with CMS across CCSQ, CCIIO, CMCS, and OEDA, we possess an understanding of the challenges and opportunities within this complex environment. Our experience is not just professional; it's also personal. As demonstrated by our insights into caregiver needs and the challenges of accessing fragmented health information, we recognize the real-world impact of these issues.

Our responses to the RFI questions reflect our commitment to advancing patient-centered care, improving data access and interoperability, and leveraging technology to create more efficient workflows for providers and payers. We highlighted critical areas such as the need for a "360-degree view" for patients and caregivers, the importance of standardizing data formats (like FHIR), the potential of Bulk FHIR data exports, and the limitations of TEFCA in its current state. We also addressed crucial aspects of digital identity, information blocking, and the need for clear APIs to engage developers.

Element Solutions is particularly excited about the prospect of contributing to initiatives that can truly transform the health technology ecosystem. Our agile and adaptable approach positions us to deliver targeted, cost-effective solutions that meet the needs of both CMS and the individuals they serve. We are enthusiastic about the opportunity to collaborate with CMS and other stakeholders to achieve the shared goal of enhancing patient access, improving care quality, and driving innovation in the health sector.

Thank you.